

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding Reasoning Paths :

- 1 . Short attention span -> Hyperactivity -> Disruptive behavior -> Behavioral issues .
- 2 . Disruptive behavior -> Impulsivity -> Difficulty following instructions -> Delayed closure of auditory nosis .
- 3 . Chronic ear infections -> Hearing impairment -> Hyperacusis -> Anxiety disorder -> Disruptive behavior .
- 4 . Family history of psychiatric disorders -> Genetic predisposition to ADHD or behavioral issues -> Hyperactivity and impulsivity -> Disruptive behavior .
- 5 . Acute otitis media -> Hearing impairment -> Behavioral abnormalities (e.g., disruptive behavior , attention deficit).

Reasoning Process :

- 1 . **Short attention span and hyperactivity**:
The boy’s symptoms include a short attention span , impulsive behavior , and frequent disruptions . These are hallmark features of attention-deficit/hyperactivity disorder (ADHD). ADHD is characterized by symptoms of inattention , hyperactivity , and impulsivity , which can lead to disruptions in daily activities and difficult adherence to rules . Behavioral therapy is often the first-line treatment for ADHD , especially in children under 12 years of age .
- 2 . **Disruptive behavior and impulsivity**:
The boy’s frequent disruptions , inability to follow instructions , and history of behavioral issues (e.g., hitting teachers) suggest impulsivity and difficulty with self-regulation . These are core features of ADHD , and behavioral therapy , such as cognitive-behavioral therapy (CBT), is highly effective in addressing these symptoms .
- 3 . **Chronic ear infections and hearing impairment**:
The boy has a history of three episodes of acute otitis media , which raises the possibility of hearing impairment . Hearing loss could contribute to behavioral abnormalities , such as disruptions and difficulty concentrating . However , the absence of tinnitus , hearing loss on examination , or a history of significant speech and language delays makes hearing impairment less likely as the primary cause of his symptoms .
- 4 . **Family history and genetic predisposition**:
The family history of major depressive disorder in the mother and Graves ' disease in the father suggests a genetic background that may predispose the child to psychiatric or neurodevelopmental disorders . ADHD , particularly with disruptive and hyperactive symptoms , could be exacerbated by this genetic context , supporting the need for behavioral therapy .
- 5 . **Acute otitis media**:
While chronic ear infections could potentially lead to hearing impairment , the boy’s symptoms are more consistent with ADHD than a direct result of otitis media . There is no mention of behavioral changes or disruptions directly tied to acute or chronic ear infections .
- 6 . **Environmental factors and developmental considerations**:
The boy’s inability to follow instructions and lack of concentration may also suggest other developmental or environmental factors , such as stress or exposure to toxic substances , though these are not explicitly mentioned in the case .

Conclusion :

Behavioral therapy is the most appropriate next step in management . The boy’s symptoms are consistent with ADHD , which is characterized by inattentiveness , hyperactivity , and impulsivity . Behavioral therapy is the first-line treatment for ADHD and is particularly effective in addressing disruptive and impulsive behaviors .

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Behavior therapy

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